



Location: SHERMAN OAKS HOSPITAL

PATIENT NAME : JUAREZ, CEJA JUAN LUIS

D-O-B : 07-25-1978

STUDY DATE : 10-12-2024

APPROVAL DATE : 10-16-2024

TO SPEAK TO A RADIOLOGIST: 877-939-7378

PATIENT ID : A311585

ACCESSION NO : A677912

REFERRING PHYSICIAN : SALCEDO RAYMUNDO

RADIOLOGIST : ADIL MAZHAR

MRI OF LUMBAR SPINE

TECHNIQUE: Multiplanar, multisequence MRI of the lumbar spine were performed without contrast in neutral position.

CLINICAL HISTORY: Back and neck pain.

FINDINGS:

Patient position: Images are evaluated in the neutral position.

Distal cord and conus medullaris: Spinal cord and conus medullaris are unremarkable.

Spondylolisthesis: Normal vertebral alignment with no evidence of spondylolisthesis.

Curvature: Straightening of the lumbar lordotic curvature. Concomitant mild levoscoliosis of the lumbar spine is seen.

Bone: Vertebral body heights are maintained.

Discs: Mild disc desiccation involving L2-L3, L4-L5 and L5-S1. Disc heights are maintained.

Findings at specific level:

T12- L1: There is no significant disc herniation or neural foraminal narrowing visualized; central canal is unremarkable.

L1- L2: There is no significant disc herniation or neural foraminal narrowing visualized; central canal is unremarkable; no sign of lateral recess stenosis.

L2- L3: A disc protrusion is identified. Spinal canal is patent. No sign of lateral recess stenosis is identified. No significant neural foraminal narrowing is identified. Disc deformity measures 2.0 mm.

L3- L4: There is no significant disc herniation or neural foraminal narrowing visualized; central canal is unremarkable; no sign of lateral recess stenosis.

L4- L5: A disc protrusion is identified. There is associated moderate bilateral neural foraminal narrowing. Concurrent mild bilateral lateral recess stenosis is seen. Spinal canal is patent. There is abutment on bilateral exiting nerve root. Annular fissure is identified. Disc deformity measures 2.5 mm.

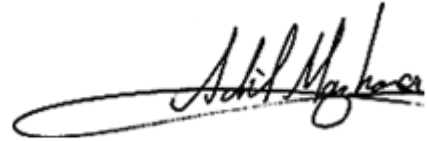
L5- S1: A disc protrusion is identified. There is associated moderate bilateral neural foraminal narrowing. Concurrent mild bilateral lateral recess stenosis is seen. Spinal canal is patent. There is abutment on bilateral exiting nerve root. Annular fissure is identified. Disc deformity measures 2.4 mm.

Impression:

1. Straightening of the lumbar lordotic curvature. Concomitant mild levoscoliosis of the lumbar spine is seen.
2. Mild disc desiccation involving L2-L3, L4-L5 and L5-S1.
3. L2-L3. A disc protrusion is identified. Disc deformity measures 2.0 mm.
4. L4-L5. A disc protrusion is identified. There is associated moderate bilateral neural foraminal narrowing. Concurrent mild bilateral lateral recess stenosis is seen. There is abutment on bilateral exiting nerve root. Annular

fissure is identified. Disc deformity measures 2.5 mm.

5. L5-S1. A disc protrusion is identified. There is associated moderate bilateral neural foraminal narrowing. Concurrent mild bilateral lateral recess stenosis is seen. There is abutment on bilateral exiting nerve root. Annular fissure is identified. Disc deformity measures 2.4 mm.



ADIL MAZHAR
RADIOLOGIST

Time Finalized: 2024-10-16 06:47:48

